QAS ACP2 pharmacology

Recall first, then check

Cover the dose column. Recall the mechanism, indication, presentation, route, adult/child dose, repeat interval, maximum and consultation trigger. Each drug has one summary card, with a link to its original QAS DTP.

What is included

Every entry in the 71-entry public DTP index with a standard ACP2 clinical regimen is included here, including regional marine-envenomation drugs, thrombolysis, obstetrics, oxygen and diluents. Adult and paediatric authority are checked separately. Consultation-required uses remain clearly labelled.

Scope selection

CCP-only, flight-only and ACP2 extended-scope regimens are excluded. Magnesium covers marine envenomation only; naloxone covers IM only; oxytocin covers IM prevention and IV PPH loading. Drugs with mixed scope retain only their ACP2 uses.

Special authorisation entries

BARPHs trial and influenza vaccine carry ACP2 badges but require separate research participation or Medical Director immuniser authorisation. They are outside this standard clinical deck. Lidocaine 1%, lidocaine 2% gel and loperamide carry ACP2 extended-scope badges and are excluded.

Sources and limits

Independent factual study notes from the Queensland Ambulance Service Drug Therapy Protocols, retrieved 12 October 2026. These are recall summaries with selected exclusions; complex eligibility checklists and exact preparation tables remain in the linked original protocols. UniSC may specify an assessment list of its own.

Adrenaline (epinephrine)

01

IM allergy, IV arrest, NEB upper airway; infusion needs approval

Adrenergic stimulation increases vascular tone, contractility and bronchodilation, addressing vasodilation, poor perfusion or bronchospasm.

PRESENTATION

1 mg/mL; 1 mg/10 mL; autoinjectors 300 or 150 micrograms.

ACP2 INDICATIONS / ROUTES

Cardiac arrest: IV. Anaphylaxis or severe life-threatening bronchospasm/silent chest: IM; adult IV infusion only with approval. Moderate/severe croup, or anaphylactic upper-airway obstruction refractory to 3 IM doses: NEB.

KEY EXCLUSIONS / PRECAUTIONS

QAS lists none. Precautions include hypertension, hypovolaemia, MAOIs and quetiapine toxicity.

ADULT ACP2 DOSES

Arrest: IV 1 mg every 3-5 min; no maximum. Anaphylaxis/severe bronchospasm: IM 500 micrograms every 5 min; no maximum. NEB 5 mg once. Adult IV infusion: approval in ALL cases; start 1-2 drops/sec (6-12 micrograms/min), adjust by 1 drop every 3-5 min to response.

PAEDIATRIC ACP2 DOSES

IM every 5 min: <6 months 50 micrograms; 6-<12 months 100; 1-<6 years 150; >=6 years 300; no maximum. Arrest IV: >=1 year (>=10 kg) 10 micrograms/kg; 6-12 months 100 micrograms; 3-5 months 70; 38 weeks gestation-2 months 50; 27-37 weeks gestation 25; <27 weeks gestation 10. Repeat every 3-5 min; no maximum. Croup/refractory upper-airway anaphylaxis NEB 5 mg once.

REMEMBER / CONSULTATION

Infusion: 1 mg into 500 mL saline = 2 micrograms/mL; dedicated line, watch extravasation and BP. NEB uses 1 mg/mL. Low-dose injections use the specified dilute preparation. No standard ACP2 IO or paediatric IV infusion regimen.

Amiodarone

300 after 3; 150 after 5; maximum 450

Prolongs cardiac action potentials and slows conduction; helps interrupt persistent ventricular arrhythmia.

PRESENTATION

150 mg/3 mL, or 50 mg/mL.

ACP2 INDICATIONS / ROUTES

Adult shockable cardiac arrest after 3 DC countershocks; ACP2 IV route.

KEY EXCLUSIONS / PRECAUTIONS

For cardiac arrest: tricyclic antidepressant overdose is contraindicated.

ADULT ACP2 DOSES

IV 300 mg over 1-2 min after 3 shocks. Repeat once: 150 mg after 5 shocks. Total maximum 450 mg.

PAEDIATRIC ACP2 DOSES

No standard ACP2 paediatric amiodarone regimen in this DTP.

REMEMBER / CONSULTATION

Class III antiarrhythmic. Count shocks and cumulative dose; monitor rhythm. Stable sustained VT and paediatric administration are CCP regimens.

Aspirin

03

Block thromboxane for the platelet's lifetime

Irreversible cyclo-oxygenase inhibition reduces thromboxane A2 and platelet aggregation at vascular injury.

PRESENTATION

White 300 mg tablet.

ACP2 INDICATIONS / ROUTES

Suspected ACS or acute cardiogenic pulmonary oedema, including pain-free patients; PO.

KEY EXCLUSIONS / PRECAUTIONS

Aspirin/NSAID allergy; bleeding/clotting disorder; current GI bleeding or peptic ulcer; <18. Precautions: previous GI bleed, anticoagulants, pregnancy, possible surgical condition.

ADULT ACP2 DOSES

Age >=18: 300 mg chewed, then a small sip of water if possible. If <300 mg taken in previous 24 h, give enough to make total 300-450 mg.

PAEDIATRIC ACP2 DOSES

Not authorised under 18 years.

REMEMBER / CONSULTATION

Irreversible antiplatelet action lasts for the platelet lifespan. Give after initial GTN if GTN is indicated. Watch GI bleeding and NSAID-triggered bronchospasm.

Box jellyfish antivenom

04

Neutralise venom rather than treat pain alone

Antivenom immunoglobulins bind Chironex fleckeri toxins.

PRESENTATION

20,000 units per ampoule; volume varies 1.5-4 mL between batches.

ACP2 INDICATIONS / ROUTES

Significant Chironex fleckeri envenomation: arrest, reduced consciousness, cardio-respiratory distress/collapse, large affected area or intractable pain; ACP2 IV.

KEY EXCLUSIONS / PRECAUTIONS

Allergy/adverse reaction. Foreign protein can cause anaphylaxis or delayed serum sickness.

ADULT ACP2 DOSES

Non-arrest: IV 20,000 units over 20 min, once. Arrest: IV 20,000 units over 2-5 min; repeat twice immediately if needed, total maximum 60,000 units.

PAEDIATRIC ACP2 DOSES

Same dose and regimen as adult: antivenom dose reflects venom burden, not body weight.

REMEMBER / CONSULTATION

Non-arrest: dilute one ampoule to final 20 mL with saline; SPRINGFUSOR 60 mL/h. Keep adrenaline available; stop infusion if reaction occurs. In arrest, start effective CPR/ALS first. Regional availability: coastal stations Rainbow Beach and north.

Cefazolin

Protect an exposed long bone from infection

A first-generation cephalosporin disrupts bacterial cell-wall synthesis, causing bacterial death.

PRESENTATION

1 g powder vial; reconstituted IV concentration 100 mg/mL.

ACP2 INDICATIONS / ROUTES

Open long-bone fracture or traumatic long-bone amputation; IV.

KEY EXCLUSIONS / PRECAUTIONS

Cephalosporin allergy; known immediate or severe penicillin/carbapenem hypersensitivity. A minor isolated penicillin rash is a precaution rather than an automatic contraindication.

ADULT ACP2 DOSES

IV 2 g over 5 min, once. Add 19 mL water for injection to 2 g powder to make final 20 mL (100 mg/mL).

PAEDIATRIC ACP2 DOSES

IV 50 mg/kg, weight rounded UP to nearest 5 kg, over 5 min, once. Source bands <=20 kg: cap 1 g; >20 kg: cap 2 g. For 1 g add 9.5 mL water to make final 10 mL; 2 g preparation as adult.

REMEMBER / CONSULTATION

First-generation cephalosporin. Contaminated or water-exposed fractures may need extra hospital coverage; hand over the dose/time. Ceftriaxone is not the open-fracture substitute.

Ceftriaxone

06

Meningococcal sepsis and selected upper GI bleeding

Third-generation cephalosporin kills susceptible bacteria; the clinical indication determines the dose and route.

PRESENTATION

1 g powder vial. IV reconstitution 100 mg/mL; IM preparation differs.

ACP2 INDICATIONS / ROUTES

Suspected sepsis: IM or IV. Upper GI haemorrhage from suspected varices in a patient with chronic liver disease/alcohol misuse: IV.

KEY EXCLUSIONS / PRECAUTIONS

Cephalosporin allergy; known immediate or severe penicillin/carbapenem hypersensitivity. Precautions include other beta-lactam allergy and calcium-containing IV solutions.

ADULT ACP2 DOSES

Sepsis: IV 2 g over 5 min, once; reconstitute with 18.8 mL water to final 20 mL. IM: 2 x 1 g doses; each 1 g + 2.4 mL water gives final 3 mL. Variceal upper GI bleed: IV 1 g over 5 min, once; 9.4 mL water gives final 10 mL.

PAEDIATRIC ACP2 DOSES

Sepsis IV: 50 mg/kg, round weight UP to 5 kg, over 5 min, once; cap 1 g in <=20 kg source bands, 2 g above 20 kg. IM: same rounded weight dosing, cap 1 g; >20 kg needs remaining dose at hospital. Split IM volumes >2 mL. Age <1 month: consultation/approval.

REMEMBER / CONSULTATION

Third-generation cephalosporin. This standard ACP2 deck uses water as IM diluent: September 2026 lidocaine 1% DTP marks ACP2 use as extended scope. Never give an IM lidocaine-containing preparation IV. Flush IV line and check calcium compatibility.

Clopidogrel

07

Block the ADP platelet signal

ADP receptor inhibition reduces platelet activation and aggregation; complements aspirin in selected reperfusion pathways.

PRESENTATION

Pink 75 mg tablet.

ACP2 INDICATIONS / ROUTES

Accepted STEMI/STEMI-equivalent pPCI when receiving interventional cardiologist requests it; or STEMI after tenecteplase with aspirin/enoxaparin; PO.

KEY EXCLUSIONS / PRECAUTIONS

Allergy; <18; modified Rankin >=4; terminal/palliative illness; suspected aortic dissection; previous intracranial haemorrhage; active bleeding other than menstruation.

ADULT ACP2 DOSES

Age >=18: pPCI 600 mg PO; after tenecteplase 300 mg PO. Swallow with small amount of water.

PAEDIATRIC ACP2 DOSES

Not authorised for paediatric patients.

REMEMBER / CONSULTATION

ADP-receptor antiplatelet. Remember the pathway: PCI loading differs from post-lysis loading. Monitor bleeding; severe renal impairment is a precaution.

Dexamethasone

Oral steroid: 0.3 mg/kg, cap 16 mg; check prior steroids

Corticosteroid suppresses arachidonic-acid-derived inflammatory mediators, helping swollen croup airways and asthma inflammation.

PRESENTATION

8 mg/2 mL injectable presentation used orally, mixed with sweet liquid.

ACP2 INDICATIONS / ROUTES

Asthma when oral medicine tolerated; croup; PO only.

KEY EXCLUSIONS / PRECAUTIONS

Allergy/adverse reaction; steroid administration within 4 h. Croup outside 6 months-8 years or asthma <6 years needs consultation.

ADULT ACP2 DOSES

Asthma: PO 16 mg once.

PAEDIATRIC ACP2 DOSES

PO 0.3 mg/kg, using 4 mg/mL product and rounding volume to nearest 0.5 mL; maximum 16 mg. Croup: 6 months-8 years; outside this range consult. Asthma: >=6 years; <6 requires consultation.

REMEMBER / CONSULTATION

Injection presentation is supplied for ORAL use; mix dose with a little sweet liquid. Use Westley Croup Score. Withhold if any steroid given in previous 4 h.

Droperidol

09

SAT >=2: 10 mg; older patient 5 mg with approval

Butyrophenone antipsychotic produces sedation to permit safer management of dangerous acute behavioural disturbance.

PRESENTATION

10 mg/2 mL, or 5 mg/mL.

ACP2 INDICATIONS / ROUTES

Acute behavioural disturbance: failed de-escalation, SAT >=2 and emergency sedation needed to prevent harm; IM or IV through an EXISTING cannula.

KEY EXCLUSIONS / PRECAUTIONS

Allergy; Lewy body dementia; Parkinson disease; previous droperidol dystonia; <8. Hypoperfusion and concurrent CNS depressants are precautions.

ADULT ACP2 DOSES

Age 16-<65: 10 mg IM/IV, repeat once after 15 min, cap 20 mg. Age >=65: ACP2 consultation/approval required; 5 mg, repeat once after 15 min, cap 10 mg. Age 13-15: approval required; 0.1-0.2 mg/kg, cap 10 mg/dose, repeat once after 15 min, cap 20 mg.

PAEDIATRIC ACP2 DOSES

Age 8-12: approval required in ALL cases; IM/IV 0.1-0.2 mg/kg, cap 10 mg/dose; repeat once after 15 min, total cap 20 mg. Under 8: contraindicated.

REMEMBER / CONSULTATION

Onset 5-15 min. Monitor airway, ventilation, BP and SAT. ACP2 needing a second dose must request CCP support where available. Suspected sepsis or physical restraint without a CCP requires approval. Do not insert IV solely for droperidol.

Enoxaparin

Under 70: 30 IV then 1 mg/kg SC; 70+: no IV

Antithrombin-mediated inhibition of factor Xa and thrombin reduces further clot formation after tenecteplase.

PRESENTATION

60 mg/0.6 mL and 100 mg/1 mL graduated syringes; both 100 mg/mL.

ACP2 INDICATIONS / ROUTES

STEMI after QAS tenecteplase, alongside aspirin/clopidogrel; age-specific IV/SC regimen.

KEY EXCLUSIONS / PRECAUTIONS

Allergy; <18; modified Rankin >=4; ischaemic chest pain >6 h; terminal/palliative illness; suspected dissection. Renal/hepatic impairment and low body weight are precautions.

ADULT ACP2 DOSES

Age 18-69: IV 30 mg, then 15 min later SC 1 mg/kg once, cap 100 mg. Age >=70: NO IV loading dose; SC 0.75 mg/kg once, cap 75 mg, with consultation/approval in ALL cases.

PAEDIATRIC ACP2 DOSES

Not authorised for paediatric patients.

REMEMBER / CONSULTATION

LMWH enhances antithrombin-mediated anticoagulation. IV: use 60 mg/0.6 mL syringe and EXPEL air bubble. SC: use 100 mg/1 mL syringe and RETAIN bubble. Check weight, age, kidney function and bleeding.

Fentanyl

Micrograms, not milligrams; count combined opioid doses

Opioid-receptor activation reduces pain signalling but can suppress ventilation; hypotension still occurs.

PRESENTATION

100 micrograms/2 mL, or 50 micrograms/mL.

ACP2 INDICATIONS / ROUTES

Significant pain: IN, IM or IV. Adult autonomic dysreflexia with SBP >160 mmHg: IM/IV. SC pain use requires consultation/approval in ALL cases.

KEY EXCLUSIONS / PRECAUTIONS

Allergy. Precautions: hypotension, respiratory failure/burns, opioid addiction and MAOIs.

ADULT ACP2 DOSES

Under 70: IV 25-50 micrograms every 5 min; IM 25-100 then up to 50 every 10 min; IN 50-100 then up to 100 every 10 min; total cap 200 micrograms. >=70/frail: IV 25 every 5 min; IM or IN 25-50 then up to 50 every 10 min; total cap 100 micrograms.

PAEDIATRIC ACP2 DOSES

Age >=1: IV 1 microgram/kg (cap 25), repeat 0.5 microgram/kg (cap 25) every 5 min, total 2 micrograms/kg. IM 1-2 micrograms/kg (cap 50), total 2 micrograms/kg. IN 1.5 micrograms/kg (cap 50), repeat once at 10 min: 1 microgram/kg (cap 50), total 100 micrograms. Under 1: approval.

REMEMBER / CONSULTATION

Adult hypotension: incremental doses <=25 micrograms IV or <=50 IM; request CCP where available. Count combined opioids: 100 micrograms fentanyl = 10 mg morphine equivalent. SC dosing is directed by consultation. No paediatric cardiogenic chest-pain use. No ACP2 IO, RSI or sedation.

Glucagon

Hypo: IM; refractory anaphylaxis: IV if available

Glucagon mobilises glycogen. Its cardiac effects do not depend on beta receptors, supporting selected refractory anaphylaxis.

PRESENTATION

1 mg powder plus solvent; reconstitute to 1 mg/mL.

ACP2 INDICATIONS / ROUTES

Symptomatic hypoglycaemia unable to self-administer oral glucose: IM. Refractory anaphylactic hypotension/shock despite 3 IM adrenaline doses and adequate fluid: IV, or IM if IV unavailable.

KEY EXCLUSIONS / PRECAUTIONS

Allergy/adverse reaction.

ADULT ACP2 DOSES

Hypoglycaemia: IM 1 mg once. Refractory anaphylaxis: IV 1 mg once; IM if no IV available. Reconstitute 1 mg with 1 mL water for injection.

PAEDIATRIC ACP2 DOSES

Weight >25 kg: 1 mg; <=25 kg: 0.5 mg, once. Route follows indication: hypo IM; refractory anaphylaxis IV or IM if IV unavailable.

REMEMBER / CONSULTATION

Raises glucose by mobilising liver glycogen; also increases cardiac activity independently of beta receptors. May be ineffective with depleted glycogen. Recheck BGL and provide oral carbohydrate when safe. Vomiting can occur.

Glucose 10%

Adult 150 then 100 mL; child 2.5 then 1 mL/kg

Restores the brain's fuel when symptomatic hypoglycaemia prevents safe oral treatment.

PRESENTATION

10% = 100 mg/mL; 250 mL bag.

ACP2 INDICATIONS / ROUTES

Symptomatic hypoglycaemia unable to self-administer oral glucose; standard ACP2 IV infusion.

KEY EXCLUSIONS / PRECAUTIONS

QAS lists none; hyperglycaemia is a precaution.

ADULT ACP2 DOSES

IV 15 g (150 mL); then 10 g (100 mL) boluses every 5 min until BGL >=4 mmol/L.

PAEDIATRIC ACP2 DOSES

IV 0.25 g/kg (2.5 mL/kg); then 0.1 g/kg (1 mL/kg) boluses every 5 min until BGL >=3 mmol/L.

REMEMBER / CONSULTATION

10% = 100 mg/mL. Confirm cannula patency and watch extravasation. Monitor BGL, consciousness and recurrence; no standard ACP2 IO regimen.

Glucose gel

Tube is 15 g; infant buccal strip is 0.2 g

Rapidly absorbed intestinal glucose provides fuel; safe ingestion is essential.

PRESENTATION

15 g glucose tube.

ACP2 INDICATIONS / ROUTES

Symptomatic hypoglycaemia when oral medication is tolerated; PO, or the specified infant buccal regimen.

KEY EXCLUSIONS / PRECAUTIONS

Unconsciousness or swallowing difficulty. Under 2 years is a precaution with a separate buccal regimen.

ADULT ACP2 DOSES

PO 15 g; repeat once at 15 min if BGL <4 mmol/L; total maximum 30 g.

PAEDIATRIC ACP2 DOSES

Age >=2: PO 15 g, repeat once at 15 min if BGL <4, cap 30 g. Age <2: BUCC 1 cm gel (0.2 g) every 5 min until BGL >=3 mmol/L.

REMEMBER / CONSULTATION

Check conscious state and ability to tolerate medication. Do not force oral gel into an unconscious patient. Recheck BGL and use a different authorised pathway when oral treatment is unsuitable.

Glyceryl trinitrate

400 micrograms every 5 min; check BP and PDE5 drugs

Vasodilation lowers ventricular filling pressure and workload, relieving selected ischaemic pain and cardiogenic pulmonary oedema.

PRESENTATION

Sublingual spray 400 micrograms/dose; infusion ampoule 50 mg/10 mL.

ACP2 INDICATIONS / ROUTES

Painful suspected ACS; acute cardiogenic pulmonary oedema; autonomic dysreflexia or Irukandji syndrome with SBP >=160 mmHg; sublingual.

KEY EXCLUSIONS / PRECAUTIONS

Allergy; SBP <100; HR <50 or >150; CVA/head trauma; sildenafil/vardenafil within 24 h; tadalafil within 48 h; riociguat. Check full DTP exclusions.

ADULT ACP2 DOSES

Sublingual 400 micrograms every 5 min as needed; no published maximum. Reassess BP and symptoms before repeat doses.

PAEDIATRIC ACP2 DOSES

Autonomic dysreflexia/Irukandji only: consultation/approval in ALL cases. No standard paediatric ACS/pulmonary-oedema regimen.

REMEMBER / CONSULTATION

Nitrate vasodilation reduces preload and myocardial oxygen demand. Inferior MI is a precaution. Ask explicitly about erectile-dysfunction/pulmonary-hypertension drugs. No ACP2 IV infusion.

Heparin

PCI adjunct: 5,000 IU IV once, or cardiologist-requested dose

Antithrombin-mediated inhibition of coagulation reduces thrombin formation, supporting selected coronary reperfusion pathways.

PRESENTATION

5,000 IU/5 mL, or 1,000 IU/mL.

ACP2 INDICATIONS / ROUTES

Accepted STEMI/STEMI-equivalent pPCI, alongside aspirin plus ticagrelor or clopidogrel; IV. Post-tenecteplase only in the specified special circumstances with consultation.

KEY EXCLUSIONS / PRECAUTIONS

Allergy, <18, Rankin ≥4, pain >12 h, terminal/palliative illness, suspected dissection, anticoagulant use, severe active bleeding.

ADULT ACP2 DOSES

pPCI: IV 5,000 IU once, or the dose requested by accepting interventional cardiologist. Special post-lysis use requires consultation/approval in ALL cases.

PAEDIATRIC ACP2 DOSES

Not authorised for paediatric patients.

REMEMBER / CONSULTATION

Antithrombin-mediated anticoagulation. Stock 5,000 IU/5 mL = 1,000 IU/mL. Check units, prior anticoagulants, bleeding and platelet history. No ACP2 aeromedical maintenance infusion.

Hydrocortisone

100 mg usually; 200 mg adult refractory anaphylactic wheeze

Glucocorticoid effects suppress inflammation; adrenal crisis instead requires replacement of deficient cortisol.

PRESENTATION

100 mg powder vial/Act-O-Vial; Act-O-Vial preparation 100 mg/2 mL.

ACP2 INDICATIONS / ROUTES

Asthma/COPD unable to tolerate oral medicine; adrenal crisis; refractory anaphylaxis with persistent wheeze after 3 IM adrenaline doses; IM/IV.

KEY EXCLUSIONS / PRECAUTIONS

Allergy; hypertension is a precaution.

ADULT ACP2 DOSES

Asthma/COPD or adrenal crisis: 100 mg IM/IV once. Refractory anaphylactic wheeze: 200 mg IM/IV once. IV doses over 1 min.

PAEDIATRIC ACP2 DOSES

Asthma ACP2: <6 years consult; printed DTP specifies >6 years 4 mg/kg, cap 100 mg, once. Exactly age 6 is not resolved in that table: consult. Refractory anaphylaxis: 4 mg/kg, cap 100 mg, once. Adrenal crisis: 0-4 years 25 mg; 5-10 years 50 mg; >10 years 100 mg. IM/IV; IV over 1 min.

REMEMBER / CONSULTATION

Act-O-Vial 100 mg/2 mL after activation. Steroid supports inflammation/adrenal replacement; onset is slower than bronchodilator or adrenaline. Check previous steroids. Do not replace first-line anaphylaxis treatment.

Hydroxocobalamin

18

Cyanide antidote: 5 g IV over 15 min

Hydroxocobalamin binds cyanide to form renally excreted cyanocobalamin, countering cyanide-related failure of cellular oxygen use.

PRESENTATION

5 g CYANOKIT vial; reconstitute with 200 mL saline.

ACP2 INDICATIONS / ROUTES

Life-threatening suspected cyanide poisoning; ACP2 adult IV infusion.

KEY EXCLUSIONS / PRECAUTIONS

Allergy. Hypertension and pregnancy are precautions.

ADULT ACP2 DOSES

IV 5 g over 15 min, once. Reconstitute 5 g with 200 mL saline 0.9%.

PAEDIATRIC ACP2 DOSES

No QAS paediatric administration authorised in this DTP.

REMEMBER / CONSULTATION

Binds cyanide to form cyanocobalamin. Skin/urine can turn red and laboratory results may be affected. Watch hypertension and infusion reactions. Assess smoke-inhalation history and provide appropriate oxygen.

Ibuprofen

200-400 mg; kidney, GI and medication exclusions matter

Nonselective NSAID suppresses prostaglandin synthesis, providing analgesia, antipyresis and anti-inflammatory effects.

PRESENTATION

200 mg oral tablet; confirm QAS presentation in source.

ACP2 INDICATIONS / ROUTES

Moderate pain from acute inflammation/tissue injury; PO.

KEY EXCLUSIONS / PRECAUTIONS

NSAID/aspirin allergy; another NSAID; GI bleed/ulcer; dehydration/hypovolaemia; renal impairment; NSAID asthma; heart failure; pregnancy; ACE inhibitor/ARB; anticoagulant or diuretic; age <13 or >65.

ADULT ACP2 DOSES

PO 200-400 mg. Do not give within 6 h of previous ibuprofen. Give with or soon after food/milk.

PAEDIATRIC ACP2 DOSES

DTP explicitly states paediatric administration is not authorised; do not infer a child dose from the <13-year exclusion.

REMEMBER / CONSULTATION

Non-selective NSAID: analgesic, antipyretic and anti-inflammatory. May accompany paracetamol when otherwise suitable. Check kidney perfusion, dehydration and interacting medicines.

Ipratropium bromide

Adults 500 micrograms; preschool 250; repeat every 20 min

Antimuscarinic action reduces vagal bronchomotor tone, adding bronchodilation through a different pathway from salbutamol.

PRESENTATION

250 micrograms/1 mL nebule.

ACP2 INDICATIONS / ROUTES

Bronchospasm; nebulised adjunct bronchodilator.

KEY EXCLUSIONS / PRECAUTIONS

Allergy; age <1 year. Glaucoma is a precaution.

ADULT ACP2 DOSES

NEB 500 micrograms every 20 min; total maximum 1.5 mg.

PAEDIATRIC ACP2 DOSES

Age 1-5: NEB 250 micrograms every 20 min, cap 750 micrograms. Age >=6: NEB 500 micrograms every 20 min, cap 1.5 mg. Under 1: contraindicated.

REMEMBER / CONSULTATION

Antimuscarinic reduces vagally mediated bronchoconstriction. Avoid eye exposure, especially with glaucoma risk. Check units: 1.5 mg = 1,500 micrograms.

Loratadine

Itchy hives, no anaphylaxis: 10 mg once

Peripheral H1 antagonism reduces histamine-mediated allergic skin symptoms; it does not reverse anaphylactic shock.

PRESENTATION

10 mg tablet.

ACP2 INDICATIONS / ROUTES

Symptomatic urticaria without anaphylaxis; PO.

KEY EXCLUSIONS / PRECAUTIONS

Allergy/adverse reaction; anaphylaxis; age <8.

ADULT ACP2 DOSES

PO 10 mg once. Do not give within 24 h of a previous antihistamine unless consultation/approval obtained.

PAEDIATRIC ACP2 DOSES

Age >=8: PO 10 mg once; same 24-h restriction. Under 8: not authorised.

REMEMBER / CONSULTATION

Second-generation peripheral H1 antagonist; less sedating. Does not treat anaphylactic airway/circulatory compromise. Watch sedation in older patients and severe hepatic impairment.

Magnesium sulphate

ACP2 magnesium: marine envenomation, 10 mmol over 20 min

Magnesium inhibits smooth-muscle contraction and has anticonvulsant/antiarrhythmic effects.

PRESENTATION

10 mmol (2.5 g)/5 mL; 2 mmol/mL.

ACP2 INDICATIONS / ROUTES

Irukandji pain unrelieved by opioids and/or SBP >160; box jellyfish envenomation unresponsive to antivenom. ACP2 IV marine-envenomation regimens only.

KEY EXCLUSIONS / PRECAUTIONS

Allergy; AV block; renal failure. Renal impairment is a precaution.

ADULT ACP2 DOSES

IV 10 mmol over 20 min; repeat once at 20 min; total maximum 20 mmol. Mix 5 mL magnesium (10 mmol) with 15 mL saline = final 20 mL; SPRINGFUSOR 60 mL/h.

PAEDIATRIC ACP2 DOSES

IV 0.1 mmol/kg, rounded UP to nearest 0.5 mmol, single cap 5 mmol; over 15 min. Repeat once at 15 min; total cap 10 mmol. Dilute required dose to final 15 mL; SPRINGFUSOR 60 mL/h.

REMEMBER / CONSULTATION

Marine use requires appropriate training and coastal station availability from Rainbow Beach north. Monitor BP, respiration and reflexes. Eclampsia, torsades, asthma and IO are CCP regimens.

Methoxyflurane

3 mL, repeat once; 6 mL/day and 15 mL/week

Low-concentration inhaled anaesthetic produces analgesia; exact mechanism is not established.

PRESENTATION

3 mL bottle plus Penthrox inhaler.

ACP2 INDICATIONS / ROUTES

Pain when able to self-administer inhaled methoxyflurane; INH.

KEY EXCLUSIONS / PRECAUTIONS

Allergy, <1 year, liver/renal disease, malignant-hyperthermia susceptibility.

ADULT ACP2 DOSES

INH 3 mL; repeat once after 20 min. Maximum 6 mL/24 h and 15 mL/7 days; consecutive-day use not recommended.

PAEDIATRIC ACP2 DOSES

Same regimen for age >=1 and able to self-administer. Under 1: contraindicated.

REMEMBER / CONSULTATION

Use Penthrox inhaler. Monitor consciousness and breathing; patient controls inhalation. Renal and hepatic history matter; track daily and weekly exposure.

Midazolam

Seizure: adult 5 mg; child 0.2 mg/kg IN/IM

Benzodiazepine increases inhibitory CNS activity; ventilation and BP can fall, especially with other depressants.

PRESENTATION

5 mg/1 mL.

ACP2 INDICATIONS / ROUTES

Generalised seizure or focal seizure with GCS <=12: adult IN/IM/IV; child IN/IM. SAT >=2 behavioural disturbance unresponsive to maximum droperidol: IM/IV only with consultation/approval in ALL cases.

KEY EXCLUSIONS / PRECAUTIONS

Allergy. Reduce/consider doses for frailty, renal failure, CCF/shock; respiratory and neuromuscular precautions.

ADULT ACP2 DOSES

Seizure: IN/IM 5 mg every 10 min or IV 5 mg every 5 min; total maximum 20 mg. Post-droperidol behavioural rescue has no fixed public ACP2 dose: consultation directs it.

PAEDIATRIC ACP2 DOSES

Seizure IN/IM 200 micrograms/kg (0.2 mg/kg), cap 5 mg/dose. Repeat HALF initial dose (cap 2.5 mg) every 10 min; total cap 10 mg. No standard ACP2 paediatric IV seizure regimen. Post-droperidol IM/IV rescue: approval in ALL cases.

REMEMBER / CONSULTATION

Benzodiazepine enhances GABA inhibition. Count earlier doses; monitor ventilation and BP. Standard CCP procedural/airway sedation doses are excluded.

Morphine

25

Milligrams; 20 mg adult cap, 10 mg older/frail

Opioid-receptor activation reduces pain perception but causes respiratory depression and vasodilation; renal disease alters suitability.

PRESENTATION

10 mg/1 mL; use authorised dilution for IV preparation.

ACP2 INDICATIONS / ROUTES

Significant pain; autonomic dysreflexia with SBP >160 mmHg; IM/IV. SC pain use requires consultation/approval in ALL cases.

KEY EXCLUSIONS / PRECAUTIONS

Allergy; kidney disease/renal failure. Hypotension, respiratory compromise, MAOIs and cardiac chest pain are precautions.

ADULT ACP2 DOSES

Under 70: IV 2.5-5 mg every 5 min; IM 2.5-10 mg then up to 5 mg every 10 min; total cap 20 mg. >=70/frail: IV 2.5 mg every 5 min; IM 2.5-5 mg then up to 5 mg every 10 min; total cap 10 mg.

PAEDIATRIC ACP2 DOSES

Age >=1: IV 0.1 mg/kg (cap 2.5 mg), repeat 0.05 mg/kg (cap 2.5 mg) every 5 min; total cap 0.2 mg/kg. IM 0.1-0.2 mg/kg (cap 5 mg); total cap 0.2 mg/kg. Under 1: approval. SC: approval in ALL cases.

REMEMBER / CONSULTATION

Adult SBP <90: incremental IV <=2.5 mg or IM <=5 mg; request CCP where available. Combine ALL opioid doses using MME (100 micrograms fentanyl = 10 mg morphine). Fentanyl preferred in renal disease/instability/suspected ACS. No paediatric cardiogenic chest-pain use; no ACP2 IO/sedation.

Naloxone

ACP2 route is IM: adult 1.6 mg once

Competes at opioid receptors, reversing respiratory and CNS depression; abrupt reversal can precipitate withdrawal.

PRESENTATION

400 micrograms/mL.

ACP2 INDICATIONS / ROUTES

Respiratory depression caused by opioid/narcotic drugs; IM.

KEY EXCLUSIONS / PRECAUTIONS

Allergy; newly born patient.

ADULT ACP2 DOSES

IM 1.6 mg once.

PAEDIATRIC ACP2 DOSES

IM 20 micrograms/kg once; cap 800 micrograms. Newly born: contraindicated.

REMEMBER / CONSULTATION

Competitive opioid antagonist. Support ventilation and observe for recurrence when opioid outlasts naloxone. Abrupt reversal may cause withdrawal, agitation, vomiting or seizures.

Olanzapine

SAT +1 and consent: oral olanzapine

Dopamine D2 blockade provides sedation and improves psychotic symptoms in a consenting, orally compliant patient.

PRESENTATION

5 mg orally disintegrating tablet.

ACP2 INDICATIONS / ROUTES

Failed de-escalation, SAT +1, orally compliant and gives consent; PO orally disintegrating tablet.

KEY EXCLUSIONS / PRECAUTIONS

Allergy; Lewy body dementia; Parkinson disease; anticholinergic overdose; previous olanzapine dystonia; age <16.

ADULT ACP2 DOSES

Age 16-<65: PO 10 mg; daily cap 20 mg. >=65/cachectic/frail: PO 5 mg; daily cap 10 mg. ACP2 >=65 requires approval. A second dose at 20 min is only on Clinical Consultation and Advice Line advice.

PAEDIATRIC ACP2 DOSES

No authorised regimen under 16 years.

REMEMBER / CONSULTATION

D2 blockade provides sedation/antipsychotic effect. Check consent, delirium and prior medication; suspected sepsis requires consultation. Monitor BP and sedation. SAT +1 differs from droperidol SAT >=2.

Ondansetron

4-8 mg adult; oral children use age bands

Serotonin-receptor blockade reduces nausea/vomiting signalling, with clinically relevant rhythm precautions.

PRESENTATION

4 mg/2 mL injection; 4 mg ODT.

ACP2 INDICATIONS / ROUTES

Significant nausea/vomiting; PO, IM or IV.

KEY EXCLUSIONS / PRECAUTIONS

Allergy, congenital long QT, apomorphine, <2 years. First-trimester use is restricted to extreme uncontrolled hyperemesis.

ADULT ACP2 DOSES

PO/IM/IV 4-8 mg; total maximum 8 mg. IV over 2-3 min. Do not administer within 8 h of previous ondansetron.

PAEDIATRIC ACP2 DOSES

PO age 2-4: 2 mg once; >=5: 4 mg once. IV >=2: 0.1 mg/kg, cap 4 mg, over 2-3 min, once. IM >=2: 0.1 mg/kg with weight rounded UP to 5 kg, cap 4 mg, once. Under 2: contraindicated.

REMEMBER / CONSULTATION

5-HT3 antagonist. Prefer ODT when appropriate; do not insert IV solely for ondansetron. All routes/ages: no dose within 8 h of previous ondansetron. Check QT, interacting medicines and hepatic impairment; first trimester restricted to extreme uncontrolled hyperemesis.

Oxygen

Most 92-96%; COPD 88-92%; arrest/CO maximum oxygen

Increases available oxygen for hypoxic tissue; excess oxygen can be harmful in selected conditions.

PRESENTATION

Medical oxygen: C cylinder 450 L; D 1,600 L; inhalation devices vary.

ACP2 INDICATIONS / ROUTES

Actual or potential systemic/local hypoxia; inhaled oxygen with device appropriate to patient and authorised airway care.

KEY EXCLUSIONS / PRECAUTIONS

Paraquat poisoning or bleomycin history with SpO2 ≥88%.

ADULT ACP2 DOSES

Arrest or CO/cyanide poisoning: maximum oxygen/100%. COPD and listed risk groups: target SpO2 88-92%. Most other presentations: 92-96%. Titrate to target and reassess.

PAEDIATRIC ACP2 DOSES

Use presentation-specific targets. Newly born: separate resuscitation CPG. Cyanotic congenital heart disease may have usual target 75-85%; confirm with carer/clinician.

REMEMBER / CONSULTATION

88-92% group also includes obesity-related hypoventilation, cystic fibrosis, neuromuscular disorders and paraquat/bleomycin exposure. CO readings can appear falsely reassuring. Diving accidents require high-flow oxygen outside this DTP. BVM oxygen needs effective ventilation.

Oxytocin

All babies delivered: 10 IU IM prevention; 10 IU IV PPH

Oxytocin raises uterine calcium-dependent contraction/tone, limiting postpartum uterine bleeding.

PRESENTATION

10 IU/1 mL ampoule.

ACP2 INDICATIONS / ROUTES

Prevent PPH in third-stage management after ALL fetuses delivered: IM. Uncontrolled primary/secondary postpartum haemorrhage: IV loading.

KEY EXCLUSIONS / PRECAUTIONS

Allergy; undelivered fetus.

ADULT ACP2 DOSES

Prevention: IM 10 IU once. PPH: IV 10 IU over 2-5 min; loading may follow the preventive IM dose.

PAEDIATRIC ACP2 DOSES

No QAS paediatric regimen published.

REMEMBER / CONSULTATION

Contracts the uterus to reduce postpartum bleeding. Confirm no undelivered fetus; monitor bleeding, BP and pulse. ACP2 maintenance infusion and IO regimens are not authorised by this DTP.

Paracetamol

15 mg/kg child; adult daily maximum 4 g

Analgesic/antipyretic effects reduce pain and fever-related distress; excessive exposure risks liver injury.

PRESENTATION

500 mg tablets; liquid 24 mg/mL.

ACP2 INDICATIONS / ROUTES

Mild/moderate pain or fever causing distress; PO.

KEY EXCLUSIONS / PRECAUTIONS

Allergy; age <1 month. Hepatic dysfunction is a precaution.

ADULT ACP2 DOSES

PO 0.5-1 g; at least 4 h after previous dose; maximum 4 g/24 h, counting all combination medicines.

PAEDIATRIC ACP2 DOSES

Age >=1 month: PO 15 mg/kg, single cap 1 g; no dose within 4 h of previous paracetamol. For 24 mg/mL liquid, use the DTP rounded weight/volume table. Under 1 month: contraindicated.

REMEMBER / CONSULTATION

Analgesic/antipyretic with little anti-inflammatory effect. Hepatic dysfunction is a precaution. Verify product concentration and total preceding doses before calculating volume.

Salbutamol

Adult 12 puffs or 5 mg NEB; preschool half these

Beta2 stimulation relaxes bronchial muscle and stimulates cellular potassium uptake.

PRESENTATION

MDI 100 micrograms/puff; nebules 2.5 or 5 mg/2.5 mL; injection 500 micrograms/mL.

ACP2 INDICATIONS / ROUTES

Bronchospasm; MDI or nebuliser.

KEY EXCLUSIONS / PRECAUTIONS

Allergy; age <1 year. Pulmonary oedema/ischaemic heart disease are precautions.

ADULT ACP2 DOSES

MDI 12 puffs (100 micrograms/puff), repeat at 10 min; no maximum. NEB 5 mg, repeat as required; no maximum.

PAEDIATRIC ACP2 DOSES

Age 1-5: MDI 6 puffs or NEB 2.5 mg. Age >=6: MDI 12 puffs or NEB 5 mg. MDI repeats at 10 min; NEB as required; no maximum. Under 1: contraindicated.

REMEMBER / CONSULTATION

Beta2 agonist relaxes bronchial smooth muscle. Watch tachycardia, tremor, hypokalaemia and metabolic acidosis. The hyperkalaemia and IV-infusion doses are CCP regimens and are excluded.

Sodium chloride 0.9%

Treat the perfusion problem; child fluid challenge needs approval

Isotonic crystalloid restores extracellular volume and provides drug dilution/flush solution; excessive volume causes overload.

PRESENTATION

0.9%: 10 mL ampoule/flush syringe, 100/500 mL bags.

ACP2 INDICATIONS / ROUTES

Poor perfusion/shock, significant burns, and dilution/flushing of authorised medicines; IV.

KEY EXCLUSIONS / PRECAUTIONS

QAS lists none. Heart/renal failure and uncontrolled haemorrhage are precautions.

ADULT ACP2 DOSES

Shock: IV titrated to physiological response; no single universal bolus for every cause. Burns >20% TBSA: 15 mL/h x TBSA percentage rounded to nearest 10%; weight >100 kg add 200 mL/h. Diluent/flush volumes follow individual DTP.

PAEDIATRIC ACP2 DOSES

Shock: ACP2 approval in ALL cases; IV 10-20 mL/kg, may repeat twice after reassessment, total maximum 60 mL/kg. Burns >10% TBSA: approval in ALL cases; use the authorised consultation plan.

REMEMBER / CONSULTATION

Isotonic crystalloid restores extracellular volume. Reassess perfusion and lungs; heart/renal failure and uncontrolled haemorrhage change the fluid decision. Standard ACP2 route is IV, not IO.

Sucrose 24%

Infant analgesia: 2 drops newborn, 10 drops older infant

Sweet oral stimulus is associated with endogenous-opioid analgesia lasting about 5-8 min.

PRESENTATION

24% sucrose, 2 mL twist-tip vial.

ACP2 INDICATIONS / ROUTES

Short procedural pain or distress from pain in infants; PO.

KEY EXCLUSIONS / PRECAUTIONS

>12 months, sucrose/fructose intolerance, glucose-galactose malabsorption, concurrent paralysis.

ADULT ACP2 DOSES

Not authorised for adults.

PAEDIATRIC ACP2 DOSES

Age 0-1 month: PO 0.1 mL (2 drops). Age 2-12 months: PO 0.5 mL (10 drops). Repeat once after 5 min. Age >12 months: contraindicated.

REMEMBER / CONSULTATION

24% solution; short analgesic effect about 5-8 min. Use age-based volume, not a hypoglycaemia dose. Check sugar intolerance/malabsorption and concurrent paralysis.

Tenecteplase

35

Weight bands 30-50 mg; age 70+ HALF dose with approval

Activates thrombus-bound plasminogen to plasmin, degrading the fibrin clot in eligible STEMI.

PRESENTATION

50 mg powder/solvent set; reconstituted 5 mg/mL.

ACP2 INDICATIONS / ROUTES

STEMI meeting the complete QAS prehospital thrombolysis CPP eligibility criteria; IV.

KEY EXCLUSIONS / PRECAUTIONS

Key absolute exclusions: allergy; <18; Rankin >=4; pain >6 h; terminal/palliative illness; dissection; active bleeding/bleeding disorder; recent major head injury; prior ICH; ischaemic stroke within 3 months; intracranial lesion. Relative: anticoagulants, surgery, prolonged CPR, pregnancy, BP >180/110 and others.

ADULT ACP2 DOSES

Age 18-69, IV over 10 sec: <60 kg 30 mg; 60-<70 kg 35; 70-<80 kg 40; 80-<90 kg 45; >=90 kg 50. Age >=70: HALF these doses, with consultation/approval in ALL cases.

PAEDIATRIC ACP2 DOSES

Not authorised under 18 years.

REMEMBER / CONSULTATION

Fibrinolytic converts clot-bound plasminogen to plasmin. Reconstituted strength 5 mg/mL. Complete the source/CPP absolute and relative contraindication checklist; this card is a recall aid, not that checklist. Increased scrutiny in patients <35 due to STEMI mimics.

Ticagrelor

Cardiologist-requested PCI ticagrelor: 180 mg

Reversible platelet ADP-receptor antagonism reduces activation/aggregation alongside aspirin and heparin.

PRESENTATION

90 mg tablet or ODT.

ACP2 INDICATIONS / ROUTES

Accepted STEMI/STEMI-equivalent pPCI when receiving interventional cardiologist requests ticagrelor; PO.

KEY EXCLUSIONS / PRECAUTIONS

Allergy; <18; Rankin >=4; pain >12 h; terminal/palliative illness; dissection; current ticagrelor/clopidogrel; prior ICH; active bleeding except menstruation.

ADULT ACP2 DOSES

Age >=18: PO 180 mg (two 90 mg tablets/ODTs).

PAEDIATRIC ACP2 DOSES

Not authorised for paediatric patients.

REMEMBER / CONSULTATION

Direct reversible ADP-receptor antiplatelet. PCI adjunct with aspirin/heparin. Check previous clopidogrel/ticagrelor, bradycardia, hepatic impairment and bleeding. This is not the DTP post-lysis clopidogrel pathway.

Tranexamic acid

37

Protect the clot: 1 g over 10 min, within the right window

Inhibits plasminogen activation/fibrinolysis, preserving formed clot during specified haemorrhage.

PRESENTATION

1 g/10 mL = 100 mg/mL.

ACP2 INDICATIONS / ROUTES

Trauma <=3 h with COAST >=3; uncontrolled primary PPH <=3 h; active post-tonsillectomy haemorrhage. ACP2 IV injection or specified adult IV infusion.

KEY EXCLUSIONS / PRECAUTIONS

Allergy/adverse reaction.

ADULT ACP2 DOSES

IV 1 g over 10 min, once. Adult infusion option: mix 1 g into 100 mL saline and administer over 10 min (approximately 3 drops/sec) using specified giving set.

PAEDIATRIC ACP2 DOSES

Trauma: consultation/approval in ALL cases; IV 15 mg/kg over 10 min, once, cap 1 g. Active post-tonsillectomy bleeding: same IV dose/rate/cap; no universal mandatory-approval statement for that listed indication.

REMEMBER / CONSULTATION

Antifibrinolytic preserves clot; it does not dissolve clot. No role in traumatic cardiac arrest. Record timing and COAST score. Rapid injection may cause hypotension; seizures are a recognised adverse effect. No ACP2 IO.

Water for injection

The drug DTP sets the diluent volume

Sterile water dissolves/dilutes medication; it is a preparation component, not a generic resuscitation fluid.

PRESENTATION

20 mL ampoule.

ACP2 INDICATIONS / ROUTES

Dissolution/dilution of medicines for authorised IM/IV administration; preparation component.

KEY EXCLUSIONS / PRECAUTIONS

QAS lists none.

ADULT ACP2 DOSES

No generic standalone dose. Use the exact added volume in the individual medication DTP.

PAEDIATRIC ACP2 DOSES

Same medication-specific preparation rule; no universal mg/kg dose.

REMEMBER / CONSULTATION

Sterile water is a diluent, not a fluid-resuscitation infusion. Added volume and final reconstituted volume can differ because powder displaces fluid. Confirm compatibility, asepsis and final concentration.